CHEST DISEASES HOSPITAL
Kuwait Ministry of Health

Lung Transplant Unit
Implementation Protocol

Clinical, Administrative, and Structural Framework

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1. Executive Summary

The Lung Transplant Unit (LTU) at the Chest Diseases Hospital (CDH), Kuwait Ministry of Health, is established as the national centre of excellence for lung transplantation and the centralised hub for the comprehensive management of all pre- and post-transplant adult patients across Kuwait. This protocol defines the structural, administrative, and clinical framework within which the LTU operates as a fully functional domestic transplant programme.

The LTU is built upon a multidisciplinary model that integrates transplant pulmonology, cardiothoracic surgery, anaesthesiology, critical care, pharmacy, and allied health professionals under a unified governance structure. It operates with the explicit mandate to deliver world-class transplant care, optimise waitlist outcomes, maintain long-term graft surveillance, and evolve in accordance with ISHLT standards.

> *This document serves as the governing implementation framework. Detailed clinical management protocols — including eligibility criteria, immunosuppression regimens, rejection grading, and antimicrobial prophylaxis — are contained in the separate CDH Lung Transplant Clinical Protocol.*

Core Directives

- Establish CDH as Kuwait's national lung transplant centre with full domestic transplant capability.
- Centralise all pre- and post-transplant care for adult Kuwaiti patients under one multidisciplinary unit.
- Maintain rigorous candidate selection and waitlist optimisation aligned with ISHLT 2021 guidelines.
- Operate a fully staffed domestic transplant programme including organ procurement, surgical, and post-operative pathways.
- Drive continuous quality improvement through structured QAPI metrics and registry benchmarking.

2. Mission, Vision & Mandate

2.1 Mission Statement

> *To provide equitable, evidence-based, and compassionate lung transplant care to all eligible adult patients in Kuwait through a nationally integrated, multidisciplinary programme centred at the Chest Diseases Hospital; to maximise graft survival and quality of life; and to continuously advance the standard of thoracic transplantation in the region.*

2.2 Vision

To be recognised as a leading lung transplant centre in the Middle East, delivering outcomes benchmarked against the world's foremost programmes, and establishing Kuwait as a self-sufficient centre of thoracic transplant expertise.

2.3 Mandate & Scope

The LTU operates under the authority of the Kuwait Ministry of Health and reports directly to the Head of Department and then to the Chief Medical Officer of the Chest Diseases Hospital. Its mandate encompasses:

- Centralised referral intake, evaluation, and listing of all adult lung transplant candidates in Kuwait.
- Coordination with MOH hospital hubs across Kuwait for the identification and referral of eligible candidates.
- Operation of a domestic lung transplant programme including donor procurement, transplant surgery, and perioperative management at CDH.
- Lifelong post-transplant surveillance and co-management of all transplant recipients residing in Kuwait.
- Coordination with the Kuwait Transplant Society and relevant national regulatory bodies.

2.4 Patient Population

> **Note:** The LTU serves adult patients aged 18 years and above. Paediatric lung transplantation is outside the scope of this unit and is referred to an appropriate paediatric centre as determined by the treating team and MOH directorate.

2.5 Regulatory & Institutional Alignment

The LTU operates in full compliance with:

- Kuwait Ministry of Health Transplant Regulations and Law No. 55 of 1987 on Human Organ Transplantation.
- Kuwait Transplant Society (KTS) protocols for deceased donor identification and organ allocation.
- International Society for Heart and Lung Transplantation (ISHLT) registry requirements and clinical guidelines.

- CDH institutional policies on patient safety, informed consent, and clinical governance.

3. Administrative Governance & Leadership

A cohesive, clearly defined leadership model is essential for navigating the complex regulatory and clinical environments inherent to solid organ transplantation. The LTU operates under an Executive Core Team with clearly delineated roles and accountability structures.

3.1 Executive Core Team

Role	Responsibilities
Head of Department (Thoracic Surgery)	Oversees the smooth running of the program and related administrative issues. Acts as a safe-guard for the program.
Head of Unit and Surgical Director (Thoracic Surgery)	Oversees donor organ procurement protocols, recipient surgical techniques, intraoperative mechanical support (ECMO/CPB), immediate postoperative surgical complications, and surgical team performance. Chairs the surgical governance committee.
Medical Director (Transplant Pulmonology)	Manages referral triage, waitlist prioritisation, immunosuppression protocols, and longitudinal post-transplant surveillance for rejection and infection. Serves as principal clinical lead for the MDT.
Transplant Administrator	Directs financial clearance, resource allocation, staffing coordination, MOH reporting, and submission of data to national and international transplant registries.
Transplant Coordinator (x2)	Responsible for the end-to-end coordination of pre- and post-transplant patient care. Facilitates evaluation workup, navigates patient difficulties, maintains communication between patients and physicians pre-transplant, and ensures continuity of daily management post-transplant. Acts as the primary point of contact for patients, families, and referring teams. Pre-emptively detects and communicates emerging clinical concerns to the responsible physician.
Senior Thoracic Surgeon (x2)	Supports the Surgical Director in all aspects of patient preparation, surgical planning, intraoperative care, and post-transplant surgical monitoring. Participates in the on-call procurement and implantation rota.
Transplant Pulmonologist (x4)	Supports the Medical Director in pre-transplant evaluation, MDT participation, and post-transplant outpatient management. Distributed across Kuwait MOH hospital hubs to ensure coverage and facilitate referrals from all regions.
Transplant Pharmacist	Manages immunosuppression therapeutic drug monitoring, adverse drug reaction surveillance, medication reconciliation, and drug interaction review for all LTU patients. Provides drug education to patients and nursing staff.
Transplant Psychologist / Psychiatrist	Conducts pre-transplant psychosocial assessments, screens for psychiatric contraindications, provides ongoing psychological support post-transplant, and participates in MDT evaluation of complex psychosocial cases.

Transplant Dietitian	Provides nutritional assessment and optimisation during pre-transplant prehabilitation and post-transplant recovery. Manages cachexia, obesity, and metabolic complications of long-term immunosuppression.
Transplant Physiotherapist / Rehabilitation Lead	Oversees the pre-transplant pulmonary rehabilitation programme and post-transplant physical recovery. Coordinates with ICU physiotherapy for early mobilisation protocols.
Transplant Social Worker	Coordinates psychosocial support, financial counselling, housing and family welfare issues, and community reintegration planning for transplant recipients.

3.2 Reporting Structure & Oversight

The LTU reports through the following governance chain:

- The Medical Director and Surgical Director report jointly to the Head of Department.
- The HOD reports to the Kuwait MOH Directorate of Specialised Medical Care.
- The Transplant Administrator maintains direct liaison with the MOH Finance and Administrative Directorate.
- The LTU collaborates with the Kuwait Transplant Society for all deceased donor procurement and allocation activities.

> *An Annual Programme Review is to be conducted by the CMO's office, inclusive of all QAPI metrics, staffing adequacy review, and registry data submission confirmation.*

4. Multidisciplinary Team (MDT) — Composition & Operating Procedures

The MDT is the central governing body for all clinical case evaluation, candidate waitlisting, and complex management decisions within the LTU.

4.1 MDT Composition

Mandatory Attendees

- Surgical Director (Thoracic Surgery)
- Medical Director (Transplant Pulmonology)
- Transplant Coordinator(s) — at least one per meeting
- Infectious Disease Physician
- Transplant Anaesthesiologist
- Transplant Social Worker

Regular Attendees

- Transplant Pulmonologists
- Transplant Psychologist / Psychiatrist
- Transplant Dietitian
- Transplant Physiotherapist / Rehabilitation Lead

Ad-Hoc Invited Participants

- Referring pulmonologists or primary care physicians presenting new cases.
- Specialised consultants (nephrology, cardiology, hepatology, rheumatology) as required by individual case complexity.

4.2 Meeting Schedule & Format

Parameter	Detail
Frequency	Weekly
Day & Time	Mondays, 08:00 AM
Venue	Chest Diseases Hospital, Lung Transplant Unit Conference Room
Quorum	Medical Director (or designated Transplant Pulmonologist) + Surgical Director + at least one Transplant Coordinator
Format	In-person; remote participation via secure video link permitted for hub pulmonologists presenting referral cases

4.3 Mandate

- Comprehensive evaluation and discussion of all new referral cases.

- Formal review of completed waitlist evaluation workups and listing decisions (List / Decline / Conditional Listing with defined criteria for re-review).
- Continuous reassessment of currently listed candidates, including status changes, deterioration, new complications, and psychosocial developments.
- Review of complex post-operative and post-discharge challenges.
- Organ offer review and accept/decline decisions for deceased donor organs.

4.4 Documentation & Output

- All MDT decisions are formally documented in a structured MDT minutes template by the Transplant Coordinator (attached at the end)
- Each case entry records: clinical summary, investigations reviewed, decision reached, rationale, responsible clinician, and follow-up action with deadline.
- Waiting list decisions are communicated in writing to the referring physician within 5 working days as part of a formal written report.
- Each patient referred will have a CDH file opened where the meeting proceedings and decisions will be saved.
- MDT minutes are retained for a minimum of 10 years as part of the transplant programme record.

5. Clinical Infrastructure & Facility Requirements

The LTU requires dedicated physical and technological infrastructure to support the full scope of a domestic lung transplant programme. Requirements are organised into two tiers: the core outpatient and post-transplant hub, and the surgical/perioperative programme components.

5.1 Dedicated Clinical Teams

Outpatient Transplant Clinic

- Dedicated clinic space with at least two consulting rooms, a lung function laboratory, and a phlebotomy/IV access suite.
- Private space for psychosocial consultations and patient education.

Dedicated Cardiothoracic ICU

- Minimum 4 beds designated for post-transplant care; isolated from general thoracic surgical cases where possible.
- Equipped for rapid deployment of Veno-Arterial (VA) and Veno-Venous (VV) ECMO.
- Continuous renal replacement therapy (CRRT) capability at each transplant bed.
- Isolation rooms with positive/negative pressure-switching capability.
- Dedicated nursing staff with specialised training in immunosuppressed patient management and early rejection recognition.

Dedicated Transplant Ward

- Step-down unit with continuous telemetry monitoring and HEPA-filtered air handling.
- Nursing staff trained in immunosuppression administration, infection surveillance, and home spirometry instruction.
- Single rooms with ensuite facilities; strict visiting protocols to minimise infection exposure.

5.2 Operating Theatre Requirements

- Designated cardiothoracic operating theatre available on an on-call basis for organ implantation.
- Heart-lung bypass pump and perfusionist availability 24/7.
- ECMO circuit primed and available within 30 minutes of activation.
- Bronchoscopy capability in theatre for intraoperative airway assessment.
- Dedicated anaesthetic team trained in single-lung ventilation and haemodynamic management of the transplant recipient.

5.3 Essential Medical Technology

Technology	Clinical Application
Ex Vivo Lung Perfusion (EVLP)	Assessment and rehabilitation of marginal donor lungs to safely expand the available organ pool. Standard protocol at 37°C with acellular perfusate.
Extracorporeal Life Support (ECLS)	Mobile and fixed ECMO circuits for intraoperative support, bridge to transplant, and management of severe Primary Graft Dysfunction (PGD Grade 3).
Advanced Bronchoscopy Suite	Routine surveillance bronchoscopy with transbronchial biopsies (TBBx), bronchoalveolar lavage (BAL), fluoroscopy-guided interventions, and airway anastomosis inspection.
Right Heart Catheterisation	Haemodynamic evaluation in pre-transplant workup and post-transplant monitoring of pulmonary haemodynamics.
High-Resolution CT (HRCT)	Pre-transplant recipient chest evaluation, donor lung assessment, and post-transplant radiological surveillance.
Home Spirometry System	Daily FEV1 monitoring for all post-transplant patients. Patient-reported data transmitted to the LTU for early detection of allograft dysfunction.

6. Domestic Transplant Programme — Operational Pathway

The LTU operates a fully integrated domestic lung transplant programme. All components of the transplant process — from donor identification through to surgical implantation and recovery — are managed within CDH with oversight from the LTU core team.

6.1 Organ Procurement Network & Donor Identification

- The LTU works in formal collaboration with the Kuwait Transplant Society (KTS) for all deceased donor organ procurement and allocation.
- All ICUs across Kuwait MOH hospitals are required to notify the KTS and LTU Transplant Coordinator immediately upon identification of a potential brain-dead donor meeting initial referral criteria.
- A dedicated on-call Transplant Coordinator is available 24 hours/day, 7 days/week to receive donor notifications and coordinate assessment.
- Donor assessment, acceptance and matching are outlined in the detailed transplant protocol.

6.2 Surgical Team Structure

Role	On-Call Responsibility
Transplant Surgeon (Lead)	Available within 60 minutes of activation call. Directs procurement team logistics, recipient implantation, and intraoperative decision-making.
Senior Thoracic Surgeon (Assist)	Available within 60 minutes. Supports implantation or leads procurement run if donor is at a distant centre.
Transplant Anaesthesiologist	Available within 60 minutes. Responsible for anaesthetic induction, haemodynamic management, and ECMO coordination.
Perfusionist (ECMO)	Available within 45 minutes. Manages CPB/ECMO circuit priming and intraoperative extracorporeal support.
Transplant Coordinator (On-Call)	Available 24/7. Coordinates activation logistics, recipient notification, theatre booking, and family communication.
ICU Consultant	Notified on activation. Prepares dedicated CTICU bed and ensures ECMO and CRRT readiness.

6.5 Programme Activation Protocol

STEP 1 — Donor Notification

- KTS contacts on-call Transplant Coordinator with donor details.
- Coordinator notifies Medical Director and Surgical Director immediately.

STEP 2 — Donor Assessment

- Surgical team dispatched for bronchoscopy and direct donor lung inspection.

- EVLP team placed on standby if ECD criteria are met.
- Accept/Decline decision communicated to KTS within 2 hours of notification.

STEP 3 — Recipient Activation

- Recipient contacted by Transplant Coordinator and instructed to present to CDH.
- Pre-operative admission and preparation initiated per standing pre-transplant order set.
- Theatre team and ICU notified; anaesthetic pre-assessment completed.

STEP 4 — Procurement & Implantation

- Procurement team departs to donor centre; cold ischaemic time tracking begins.
- Target cold ischaemic time: ≤ 6 hours (bilateral) / ≤ 8 hours (single lung).
- Implantation proceeds per standard surgical protocol; ECMO initiated as per intraoperative decision matrix.

STEP 5 — ICU Handover

- Structured verbal and written handover from surgical team to CTICU team at the bedside.
- Immediate post-operative orders activated; lung-protective ventilation commenced.

7. Clinical Pathways & Protocols

This section provides a high-level overview of the five clinical phases. For full detail — including eligibility criteria, disease-specific thresholds, immunosuppression protocols, rejection grading, antimicrobial prophylaxis, and follow-up schedules — please refer to the CDH Lung Transplant Clinical Protocol (separate document). The Structured Referral Form should be used by all referring physicians.

Phase I — Candidate Evaluation

A rigorous, evidence-based evaluation process is enforced to ensure appropriate organ stewardship and candidate safety.

- **Standard workup:** Diagnostics:
  - Pulmonary function tests (PFTs): spirometry, lung volumes, DLCO.
  - High-resolution CT (HRCT) chest.
  - Quantitative V/Q scan.
  - Echocardiography and right heart catheterisation.
  - 6-minute walk test (6MWT).
  - Full metabolic, haematological, immunological, and microbiological panel.

- Contraindications are assessed per ISHLT 2021 guidelines and CDH Clinical Protocol (refer to Protocol Sections 3–5).
- All evaluation cases must be presented to the weekly MDT for formal listing decision.

Phase II — Waitlist Optimisation & Prehabilitation

- **Prehabilitation:** Pulmonary Rehabilitation:
  - Mandatory supervised exercise programme, minimum 3 sessions per week.
  - Baseline and reassessment 6MWT performed every 3 months.

- **Nutrition:** Nutritional Optimisation:
  - BMI target 18.5–35 kg/m²; dietitian-led intervention if outside range.

- **Microbiology:** Infection Surveillance:
  - Routine sputum surveillance for multi-drug resistant organisms (Burkholderia cenocepacia, Mycobacterium abscessus) at listing and every 6 months.

- **Status review:** Waitlist Status Review:
  - All listed candidates reviewed at MDT at least quarterly, or sooner if clinical deterioration is suspected.
  - Inactive status, delisting, or urgency upgrade decisions documented at MDT with rationale.

Phase III — Perioperative & Surgical Management

- Donor Assessment: Evaluation per Section 6.2 criteria.
- Intraoperative Strategy: Protocolised decision matrix for VA ECMO versus CPB based on haemodynamic tolerance during pneumonectomy.
- Induction Immunosuppression: Basiliximab (non-depleting IL-2 receptor antagonist) administered at induction; refer to CDH Clinical Protocol for full induction and maintenance regimen.
- Target cold ischaemic time ≤ 6 hours bilateral; ≤ 8 hours single lung.

Phase IV — Postoperative ICU Protocols

- Ventilator Weaning: Lung-protective ventilation (Vt 6 ml/kg IBW, plateau pressure ≤ 25 cmH_2O). Target extubation within 24–48 hours where haemodynamically stable.
- PGD Management: Staged protocol — iNO for moderate PGD; VV ECMO for Grade 3 PGD at 72 hours. Refer to CDH Clinical Protocol for full PGD grading and management algorithm.
- Immunosuppression: Calcineurin inhibitor (Tacrolimus) trough levels therapeutic by day 3–5; combined with Mycophenolate Mofetil and tapering corticosteroids.
- Prophylaxis: Antimicrobial, antifungal, and antiviral prophylaxis per CDH Clinical Protocol.

Phase V — Long-Term Surveillance

- Surveillance Bronchoscopy: Protocol bronchoscopy with TBBx and BAL at 1, 3, 6, 12 months post-transplant, then annually to 5 years (or as clinically indicated thereafter).
- CLAD Monitoring: Daily home spirometry. FEV1 decline ≥ 10% from post-transplant baseline triggers urgent clinic review within 5 working days.
- Outpatient Follow-Up: Weekly for first month; fortnightly months 2–3; monthly months 4–12; then quarterly from year 2 (refer to CDH Clinical Protocol for full schedule).
- Rejection: All biopsy-proven rejection episodes (acute cellular, chronic lung allograft dysfunction) managed per CDH Clinical Protocol grading and treatment algorithm.
- Long-term outcomes are entered into the ISHLT registry annually by the Transplant Coordinator.

8. Training, Accreditation & Capacity Building

The LTU maintains a structured training and competency framework to ensure all staff meet the standards required for a safe and accredited transplant programme.

8.1 Staff Training Requirements

- All transplant nurses (ward and ICU) must complete a recognised transplant nursing competency programme prior to independent practice.
- Transplant physicians and surgeons must have completed subspecialty training in transplant pulmonology or thoracic transplant surgery at an accredited international center.
- Annual competency assessments are required for all LTU clinical staff.

8.2 Simulation & Drills

- Annual simulation exercises for organ activation, ECMO initiation, and acute rejection escalation.
- Quarterly nursing drills for immunosuppressed patient emergency management.

8.3 Accreditation Pathway

- The Transplant Administrator coordinates all documentation requirements for institutional and MOH accreditation submissions.

9. Quality Assurance & Performance Improvement (QAPI)

A continuous data-driven review process is critical for maintaining accreditation, driving clinical excellence, and benchmarking against international standards (International Society for Heart and Lung Transplant). The QAPI programme is led by the Medical Director with Transplant Administrator support.

Domain	Key Performance Indicators (KPIs)	Review Frequency
Surgical Outcomes	30-day mortality; 1-year survival; reoperation for bleeding; PGD Grade 3 at 72 hours; cold ischaemic time compliance (≤06h bilateral).	Monthly
Medical Management	Acute cellular rejection rate at 1 year; CLAD incidence at 3 years; CMV viraemia incidence; invasive fungal infection rate; tacrolimus therapeutic trough achievement by day 5.	Quarterly
Waitlist Management	Time from referral to MDT evaluation (≤14 days target); time from MDT decision to listing; waitlist mortality rate; proportion of candidates completing prehabilitation.	Quarterly
Long-Term Graft Survival	1-year, 3-year, and 5-year graft survival; BOS-free survival at 3 years; patient quality of life score (SF-36 or equivalent) at 1 and 3 years post-transplant.	Bi-annually
Operational Efficiency	ICU length of stay; total hospital length of stay; 30-day unplanned readmission rate; bronchoscopy surveillance compliance at 1 and 3 months.	Quarterly
Patient Safety	Medication errors per 100 patient days; bloodstream infection rate per 1000 catheter-days; surgical site infection rate.	Monthly
Benchmarking	Comparison of 1-year and 3-year survival against ISHLT registry medians for comparable centre volume and diagnostic mix. Reported at Annual Programme Review.	Annual

Note: All KPI data are collated by the Transplant Coordinator and presented to the MDT at regular intervals. Adverse outcome trends trigger a formal Root Cause Analysis (RCA) within 30 days, with corrective action plans documented and reviewed at the subsequent QAPI session.

10. Document Control

Version	Date	Author(s)	Summary of Changes
1.0	2024	LTU Core Team	Initial draft
2.0	2025	Medical Director / Surgical Director	Expanded governance, updated eligibility criteria, QAPI revision
3.0	2026	LTU Core Team	Comprehensive revision: Mission/Vision, domestic programme pathway, expanded team roles, updated QAPI, training framework

Appendix

CHEST DISEASES HOSPITAL
Lung Transplant Unit | Kuwait Ministry of Health

MDT Transplant Decision Form

LTU Case No.: _____

MDT Date: _____

MRN: _____

A. PATIENT IDENTIFICATION

Full Name	Date of Birth	Age	Gender	Civil ID
Referring Physician	**Referring Institution**	**Date of Referral**	**Nationality**	

B. CASE TYPE (SELECT ONE)

☐ New Referral ☐ Waitlist Review ☐ Organ Offer Review ☐ Post-Tx Review ☐ Complex / Other

C. CLINICAL SUMMARY

Primary Diagnosis	Disease Duration	Underlying Aetiology / Subtype

Functional Parameters (most recent)

FEV_1 (% pred)	FVC (% pred)	DLCO (% pred)	TLC (% pred)	6MWT (m)	SpO_2 Rest	SpO_2 Exertion

O_2 Requirement	NYHA / mMRC Class	Pulm. Artery Pressure (mPAP)	WHO/UNOS Dx Category

D. MULTIDISCIPLINARY EVALUATION SUMMARY

Domain	Findings / Concerns	Domain	Findings / Concerns
Cardiac (Echo / RHC)		Renal Function	
Hepatic Function		Haematology	
Microbiology / MDR Status		Oncology Screen	
Nutritional Assessment (BMI)		Functional / Rehab Status	
Psychosocial / Psychiatric		Social Support	

Radiology (HRCT / V-Q)		**Immunology / PRA / DSA**	

E. CONTRAINDICATION SCREENING

Absolute Contraindication	Present?	Relative Contraindication / Concern	Present?
Active or recently treated malignancy (<2 yrs, histology-dependent)	☐ Yes ☐ No	Severe osteoporosis with vertebral fractures	☐ Yes ☐ No
Uncontrolled extrapulmonary infection	☐ Yes ☐ No	BMI >35 kg/m² or <17 kg/m²	☐ Yes ☐ No
Active substance abuse or non-adherence	☐ Yes ☐ No	Mechanical ventilation (non-bridging)	☐ Yes ☐ No
Severe irreversible dysfunction of major organ (renal, hepatic, cardiac)	☐ Yes ☐ No	Highly resistant / untreatable pathogen (e.g., B. cenocepacia, M. abscessus)	☐ Yes ☐ No
Absence of adequate psychosocial support	☐ Yes ☐ No	Significant psychiatric disorder — undertreated	☐ Yes ☐ No
Documented, persistent non-adherence to medical therapy	☐ Yes ☐ No	Frailty / poor functional reserve (SPPB ≤6)	☐ Yes ☐ No

F. MDT DISCUSSION SUMMARY

Key points raised, concerns discussed, and specialist input noted:

G. MDT DECISION

☐ LIST FOR TRANSPLANT	☐ CONDITIONAL LISTING	☐ DEFER — Further Workup	☐ INACTIVE STATUS	☐ DECLINE / DELIST
Proceed to active listing	*Conditions must be met first*	*Pending outstanding investigations*	*Remains listed; suspended*	*Not suitable for transplantation*

If Listed — Transplant Specification

Procedure Type	Side (if SLT)	Priority / Urgency	Recipient Size Range
☐ Bilateral (BLT) ☐ Single (SLT)	☐ Right ☐ Left ☐ Either	☐ Standard ☐ High ☐ Urgent	TLC range: ____ to ____ L

Special donor considerations / restrictions: _____

If Conditional / Deferred / Declined — Rationale & Conditions

Re-review date / trigger: _____

H. ACTION ITEMS & FOLLOW-UP

Action Required	Responsible Person	Target Date	Completed
			☐
			☐
			☐
			☐

I. NEXT MDT REVIEW

☐ 1 Week	☐ 1 Month	☐ 3 Months	☐ 6 Months	☐ Specific Date: _____

J. MDT ATTENDANCE & SIGNATURES

MDT Member	Role / Specialty	Signature / Initial	Attended?
Surgical Director	*Thoracic Surgery (Chair)*		☐ Present ☐ Remote
Medical Director	*Transplant Pulmonology*		☐ Present ☐ Remote
Transplant Coordinator	*Transplant Coordination*		☐ Present ☐ Remote
	Pharmacy		☐ Present ☐ Remote
	Infectious Disease		☐ Present ☐ Remote
	Transplant Anaesthesiology		☐ Present ☐ Remote
	Social Work / Psychology		☐ Present ☐ Remote
	Additional / Invited Specialist		☐ Present ☐ Remote

K. CHAIRPERSON SIGN-OFF

MDT Chair Name (Print)	Signature	Date	Time

This form must be filed in the patient's medical record immediately following the MDT meeting. A copy is to be retained in the LTU registry database.